



Nursing Practice Guideline

Subcutaneous Infusions (Syringe Drivers) and Subcutaneous Fluid Administration

Reference #: NPG32

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	Nursing

Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to Subcutaneous Infusions (Syringe Drivers and Subcutaneous Fluid Administration). It ensures the delivery of a uniform, integrated multidisciplinary approach with management that is consistent with best practice.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGH Mission & Values	☒	Staff Safety	☒
Other:	☐		

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Principles

Observe the five moments for hand hygiene and aseptic technique when inserting a subcutaneous catheter. Follow the risk assessment outlined in the SCGOPHCG Infection Prevention and Management Manual -Aseptic Technique Policy.

All prescriptions written within SCGOPHCG are to meet the minimum requirements set under the Medicines and Poisons Act 2014 and Medicines and Poisons Regulations 2016, in addition to best practice requirements. It is a legislative requirement that a valid prescription must exist before Schedule 4 and Schedule 8 medications are administered.^{8,9,10,11} A nurse who is registered as a

nurse practitioner with the Nurses and Midwives Board of WA and is working in a designated area may prescribe medications from approved clinical protocols.

1.0 Insertion of Subcutaneous Catheter

1.1 Nursing Management

- SCGOPHCG uses the **BD Saf-T-Intima™ with Y Adapter** subcutaneous catheter. Also known as a subcut butterfly. Refer to **Diagram 1**. (For the purpose of this Policy it will be referred to as a subcut butterfly)
- A subcut butterfly should be considered if patients are receiving regular PRN medication administration.
- Discuss with the ward pharmacist if unsure if there is a contraindication for medication to be administered via the subcutaneous route.
- The subcut butterfly can remain in place for up to 7 days – please ensure that the insertion date is recorded on the inserted subcut butterfly and documented in the medical notes. Some medications are irritating, and site rotation may need to be more frequent.^{1,3,5,6}
- For a subcut butterfly used for PRN medication the insertion site should be assessed daily and documented in the medical record.
- When choosing an insertion site consider the patient's mobility, comfort and ease of access. For ambulatory patients consider the abdomen or chest. For agitated or distressed patients, the scapula is preferred site. Refer to **Diagram 2** for preferred insertion sites.
- See **Table 1** for unsuitable sites for subcut butterfly placement.
- If the site becomes inflamed, painful or leakage of the infused fluid occurs, remove needle and re-site in another location.^{1,3,6,7} Document in medical record and nursing care plan.
- When administering medication, a saline flush is only required post medication administration not pre administration.
- Do not remove the clamp from the subcut butterfly as it is required for set up and syringe changes.

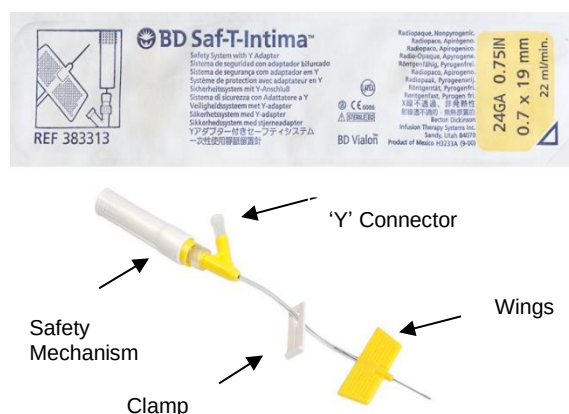


DIAGRAM 1 BD Saf-T-Intima™

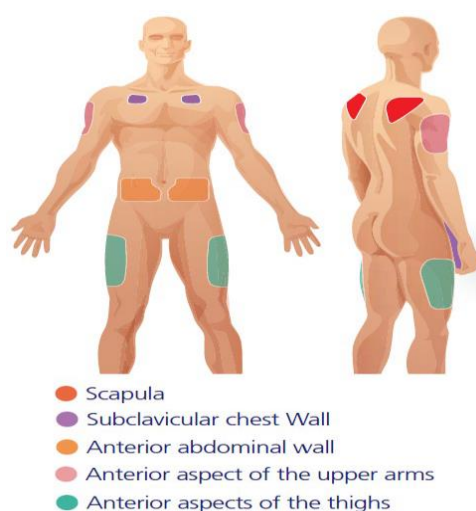


DIAGRAM 2 Insertion Sites^{1,3}

Table 1. Sites not suitable for subcutaneous catheter placement.^{1,3}

Site	Reasons why unsuitable
Skin Folds	• Device cannot be safely secured. • Infusion site not easily observed. • Potential for impaired absorption
Limb Oedema/ Lymphoedema	• Infection Risk • Impaired absorption
Bony Prominence	• Reduced subcutaneous tissue. • Impaired absorption • Device cannot be safely secured
Previously Irradiated Skin	• Impaired blood supply – reduced absorption • Infection risk • Dry/delicate skin
Near joints	• Potential for dislodgment • Uncomfortable for patient
Infected/broken skin	• Potential for infection

1.2 Procedural Information

Insertion of subcutaneous catheter

Equipment

- Clean disposable kidney dish
- 24g Saf-T-Intima™ catheter (see Diagram 1)
- Chlorhexidine 2% in 70% alcohol swab stick /solution
- Sterile transparent semi-permeable dressing^{1,3} (e.g. Opsite 3000™).
- Fixomul™ (if required reinforce edges with a border)
- 1 x 10ml sodium chloride 0.9%
- Needle Free Valve Port
- Personal Protective Equipment (PPE)

Preparation

1. Gather equipment.
2. Perform hand hygiene.
3. Using aseptic non-touch technique, remove the plastic cap at the 'Y' connection and add needle free valve port.
4. Prime needle free port and catheter with 0.9% sodium chloride (normal saline).

At patient bedside

1. Perform hand hygiene.
2. Select appropriate insertion site and cleanse with Chlorhexidine 2% in 70% alcohol swab stick/ solution and allow to air dry.
3. Perform hand hygiene and don PPE.
4. Grip the bubbled surface of the yellow wings of the subcut butterfly between the thumb and the forefinger.
5. Remove and dispose of plastic needle cap.
6. Ensure the needle bevel edge is orientated with the point towards the skin. Insert the needle at a 45-degree angle whilst holding the skin between the thumb and the forefinger³.
7. Place the transparent semi-permeable dressing over the insertion site to cover and secure the needle^{1,3}.

8. Press gently on the winged area and withdraw the introducer by gently pulling the safety mechanism. Dispose into sharps container.
9. If required place Fixomul™ to secure edges.
10. Complete and place "Date of Insertion" sticker on the edge of the dressing.
11. Dispose of equipment.
12. Perform hand hygiene.
13. Document date and insertion site in the nursing care plan and in the medical record.

2.0 Subcutaneous Infusions - Syringe Driver

2.1 Medical Requirements

A valid prescription must be written on the Subcutaneous Infusion Prescription and Monitoring Form.

2.2 Safety Information

SCGOPHCG uses the BD BodyGuard T syringe driver for subcutaneous medication administration. It is calibrated in mls per hour and set for a standard 24hr continuous infusion.

Use a 20ml BD Plastipak luer lock syringe.

Syringe must be labelled with subcutaneous additive label¹². Place the additive label so graduations on the syringe scale remain visible.¹



*To be placed on
Extension tubing*

The syringe driver must be placed within the locked box. Key located on Schedule 8 keys.

Prior to MRI consider PRN dose of medication as syringe driver must be disconnected and contents appropriately discarded. Syringe Driver will need to be set up from start when patient returns to the ward.

Syringe Drivers are NOT waterproof. Please protect whilst showering. Do not disconnect.

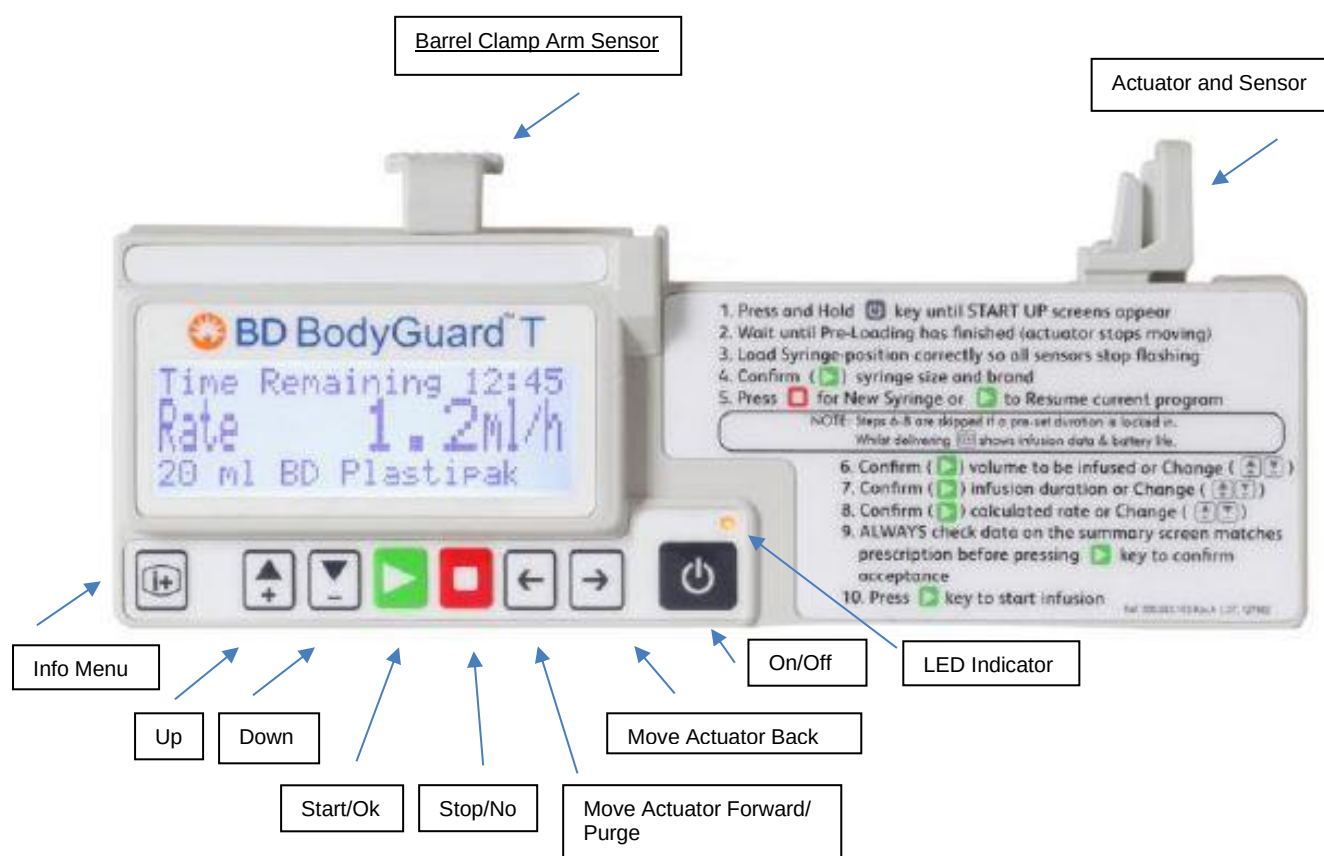
2.3 Nursing Management

- **Location of equipment**
 - **SCGH:** Syringe drivers are located in Palliative Care Office. Please contact the office to request a pump. A patient identification sticker will be required. Out of hours contact after-hours CNS.
 - **OPH:** Syringe drivers are located on Wards 2, 3, 4,5 and 7. See your CNM.
- Any change of medication and/or dosing requires both a new syringe and a new extension line.¹
- Syringes must be changed at least every 24 hours.
- Extension tubing can remain in place for up to 72 hours.
- Do NOT manually prime extension set, use the syringe driver prime function. See procedural information for further detail.
- Syringe drivers require a dedicated subcut butterfly. **The subcut butterfly should be primed with the prescribed medication on syringe driver initiation.**¹
- Ensure a second subcut butterfly for PRN medication administration. See Subcutaneous Catheter Insertion section.
- When placing the additive label on the syringe ensure it does not obscure the visual scales or interfere with the mechanism of the pump.
- All new prescriptions should be commenced within 1 hour, do not wait for an existing infusion to complete.

- Patients are NOT to be routinely discharged with a syringe driver. The palliative care team will assist with discharge planning if this is advised.
- For new infusions or medication/dose changes the patient's symptoms and insertion site should be assessed 1 hour after the infusion has been commenced and then commence 4 hourly observations as per the Prescription and Monitoring form MR 815.
- If Battery life is less than 25% change to a new battery.
- The Syringe Driver screen should be locked after commencement of the infusion. To access the information required for pump checks press the blue information button.
- To LOCK/UNLOCK Syringe Driver press and hold the Blue Information key. Do not release the blue key until you hear the tone.

2.4 Procedural Information

BD BodyGuard T Syringe Driver (OPH and SCGH)



1. Syringe Driver commencement (with initial insertion of Subcutaneous Catheter)
2. Syringe Change with Medication and/or dosing change
3. Syringe Change only (no change to medication or dosing)

1. Syringe Driver commencement (with initial insertion of Subcutaneous Catheter)

Equipment

- Syringe driver and Perspex locked box
- Clean disposable kidney dish
- 24g Saf-T-Intima™ catheter (see Diagram 1)
- 20ml luer lock BD Plastipak syringe
- Drawing up needle

- 75cm luer lock extension set.
- Prescribed medications/diluents as per prescription form (MR 815)
- Additive label
- Extension tubing label.
- Large 70% Isopropyl alcohol swab
- Sterile transparent semi-permeable dressing^{1,3} (e.g., Opsite 3000™). You may need to reinforce edges with Fixomul.
- Personal Protective Equipment (PPE)

Preparation

- Gather all required equipment.
- Perform hand hygiene.
- Prepare medication in syringe as per prescription. (Please ensure medication is drawn up first then followed by diluent **up to 18 mls**. Place additive label on syringe avoiding visual scale.
- Using aseptic non touch technique, attach the extension tubing to 'Y' connection of the subcut butterfly (**See Diagram 1.**), then to the 20ml BD Plastipak syringe. Ensure date label is attached to extension tubing.

Loading and Priming the Syringe Driver

1. Perform hand hygiene.
2. Unlock Perspex box. Press and hold the Power button on the Syringe Driver until the start-up screen appears.
3. Wait until Pre-loading has finished (the actuator stops moving).
4. To load syringe, lift and turn barrel clamp handle, insert prepared syringe into the correct position. If the actuator is not in the correct position to accommodate the syringe, use the buttons to adjust. 
5. Close barrel clamp handle. Check the display reads 20ml BD Plastipak. Press  button.
6. Check the Duration is set for 24:00, Press the  button.
7. When screen displays "Start Infusion," Press the  button. (The line and subcut butterfly need to be primed)
8. Press the  Button
9. The screen will display "Purge", Press  button to confirm.
10. Hold down the  to prime the line and subcut butterfly; release the button when the line and subcut butterfly have been primed and medication is visible at the tip of the needle. Approximately 1.3 ml of fluid.
Wait for the screen to return to showing "20ml BD Plastipak". Press  button.
11. Insert subcut butterfly as per instruction in the Procedure for Insertion of Subcutaneous Catheter commencing from step 7. (See Page 4).
12. Press  for new syringe.
13. Press  button to confirm.
14. Press  button to "Start Infusion".

15. Screen should display "Pump Delivering".
16. Lock pump by pressing and holding blue information button. Ensure Perspex box is locked.
17. Ensure Extension tubing label has been attached with date due for change.
18. Perform hand hygiene.
19. Document catheter insertion in Nursing Care Plan and medical notes.
20. Commence the Subcutaneous Infusion Prescription and Monitoring Form. Commence observations assessing the insertion site at 1 hour after the infusion has been commenced and then 4 hourly observations.

2. Syringe Change with Medication and/or dosing change

Equipment

- Clean disposable kidney dish
- 20ml luer lock BD Plastipak syringe
- Extension tubing
- Extension tubing label
- Drawing up needle
- Prescribed medications/diluents as per prescription form (MR 815)
- Additive label
- Large Isopropyl 70% alcohol large wipe
- Personal Protective Equipment (PPE)

Preparation

1. Gather equipment.
2. Perform hand hygiene.
3. Prepare medication in syringe as per prescription. (Please ensure medication is drawn up first then followed by diluent **up to 18 mls**.)
4. Attach extension tubing and tubing label using non touch technique.
5. Place additive label on syringe avoiding visual scale.

At Patient bedside

1. Perform hand hygiene.
2. Unlock Perspex box and unlock pump by holding down the blue information key.
3. Turn off pump to allow pump to reset.
4. Clamp subcut butterfly.
5. Remove old syringe and extension tubing from patient and pump. Make sure to replace the barrel clamp handle to original position.
6. Press and hold the Power button on the Syringe Driver until the start-up screen appears.
7. To load syringe, lift and turn barrel clamp handle, insert prepared syringe into the correct position. If the actuator is not in the correct position to accommodate the syringe, use the   to move the actuator to the correct position. Close barrel clamp handle.
8. Press  to confirm syringe type – 20ml BD Plastipak
9. Check duration is set for 24:00. Press  button to confirm.
10. When screen displays "Start Infusion," Press the  button. (The new line requires priming)
11. Press the  Button
12. The screen will display "Purge", Press  button to confirm.

13. To prime the line, hold down , and release the button when the line has been primed and medication is visible at the tip of the line. Approximately 1.3 ml of fluid.

Wait for the screen to return to showing “20ml BD Plastipak”. Press  button.

14. Clean ‘Y’ connector using a Large 70% Isopropyl alcohol wipe.
15. Attach extension tubing to patient
16. Unclamp subcut butterfly.
17. Press  for new syringe.
18. Check duration is set for 24:00. Press  button to confirm.
19. Pump will ask “Start Infusion?” Press  button to confirm.
20. Pump will commence. It will display rate and “Pump Delivering”.
21. Lock pump by pressing and holding blue information button.
22. Ensure extension tubing label has been attached with next date due for change.
23. Perform hand hygiene.
24. Record commencement of new syringe on the Subcutaneous Infusion Prescription and Monitoring form. Commence observations Assess insertion site at 1 hour after the infusion has been commenced and then commence 4 hourly observations.
25. Dispose of equipment. Ensure previous syringe and line is disposed of within the sharps container and discarded contents recorded on the Subcutaneous Infusion Prescription and Monitoring form.

3. Syringe Change Only – no change to medication or dosing

Equipment

- Clean disposable kidney dish
- 20ml luer lock BD Plastipak syringe
- Drawing up needle
- Prescribed medications/diluents as per Subcutaneous Infusion Prescription and Monitoring form (MR 815)
- Additive label
- Personal Protective Equipment (PPE)

Preparation

1. Gather equipment.
2. Perform hand hygiene.
3. Prepare medication in syringe as per prescription. (Please ensure medication is drawn up first then followed by diluent **up to 18 mls.**
4. Place additive label on syringe avoiding visual scale.

At Patient Bedside

1. Perform hand hygiene.
2. Unlock pump by holding down the blue information key. Open Perspex box.
3. Turn off pump to allow pump to reset.
4. Remove old syringe and attach new syringe to extension tubing using non-touch technique. Make sure to replace the barrel clamp handle to original position.
5. Press and hold the Power button on the Syringe Driver until the start-up screen appears.

6. To load syringe, lift and turn barrel clamp handle, insert prepared syringe into the correct position. If the actuator is not in the correct position to accommodate the syringe, use the

  buttons to move the actuator to the correct position. Close barrel clamp handle.

7. Press  to confirm syringe type – 20ml BD Plastipak.
8. Check duration is set for 24:00. Press  button to confirm.
9. Pump will ask “Start Infusion?” Press  button to confirm.
10. Pump will commence. It will display rate and “Pump Delivering”. Check time remaining and rate correct.
11. Lock pump by pressing and holding blue information button. Ensure Perspex box is locked
12. Perform hand hygiene.
13. Record commencement of new syringe on the Subcutaneous Syringe Driver Prescription and Monitoring form.
14. Dispose of equipment. Ensure previous syringe is disposed of and discarded contents recorded on the Subcutaneous Prescription and Monitoring form. (Subcutaneous Syringe Driver MR 815).

Troubleshooting the BD BodyGuard T Syringe Drivers

FAULT	POSSIBLE CAUSE	ACTION
The pump will not start.	- No battery present. - Battery inserted incorrectly. - Battery is depleted/very low	- Fit a new battery. - Realign battery terminals. - Fit a new battery.
Infusion ended early/going too quickly	- Wrong syringe brand confirmed during set up. - Pump faulty	- Check set up correct. Any concerns contact Palliative Care CNC/CN. - Service calibration required.
The pump has stopped before emptying syringe	- Exhausted battery - Faulty pump	- Fit new battery, turn pump on, and confirm syringe size and brand select to resume infusion. - Return for service.

3.0 Subcutaneous Infusions – Continuous Ambulatory Drug Delivery (CADD) pump

3.1 Medical Requirements

A valid prescription must be written on the Subcutaneous Infusion Prescription and Monitoring form – CADD

3.2 Safety Information

SCGOPHCG uses the CADD Solis pumps. The CADD pumps can be configured to deliver medications as prescribed on the form.

The CADD pumps use a cassette attached to the pump for storage and delivery of medications. This cassette is made up in Aseptic Services at SCGH. Aseptic services require 24-48 hours' notice for cassettes to be made.

The Subcutaneous additive label should be attached to the cassette. Check the contents of the cassette against the medical order and complete the additive label.¹²

The extension tubing must be labelled with a subcutaneous label. The line does not need to be dated as the extension line is replaced when the medicine order changes, or the cassette is replaced.

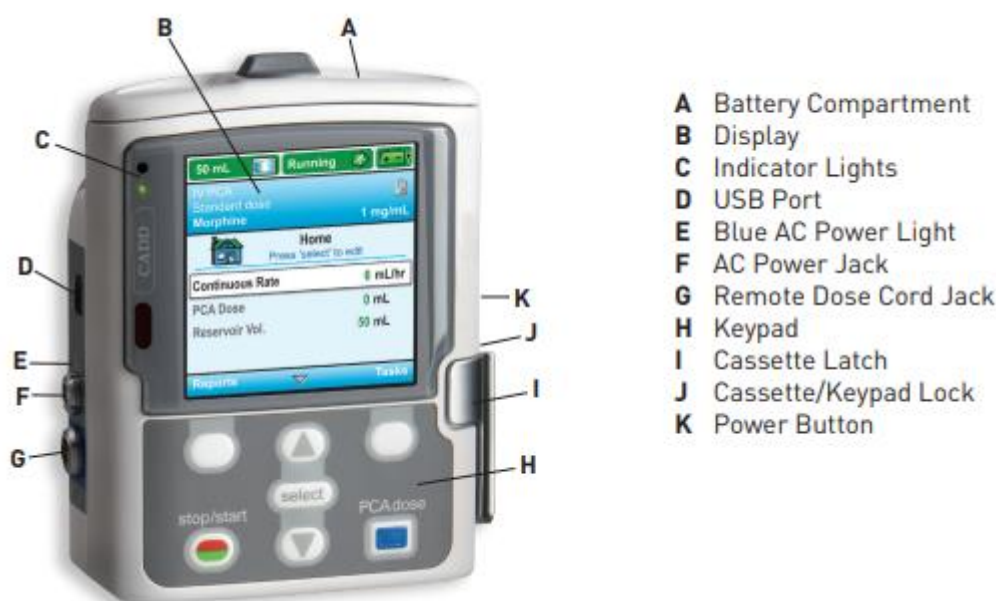
The CADD Solis pumps are not waterproof. Please protect whilst showering, do not disconnect.

3.3 Nursing Management

- Location of Equipment – CADD Solis pumps are in the Palliative Care Office at SCGH. If a patient at OPH requires a pump this will be provided by the Palliative Care CNC.
- CADD Solis pumps require a dedicated subcutaneous (subcut) butterfly. The subcut butterfly will be primed with the prescribed medication on CADD pump initiation. This will be performed by the Palliative Care Team.

3.4 Procedural Information

CADD Solis Pump



Only patients under the care of the Palliative Care team can have a CADD Solis pump for subcutaneous infusion of medications.

The prescription for medications can only be completed by a member of the Palliative Care medical team and this will be documented on the Prescription and Monitoring Form. Commencement of all CADD Solis pumps and ordering of cassettes will be done by the Palliative Care team.

Only the Palliative Care team can make changes to the CADD Solis prescription.

Nursing staff are only expected to perform observations as listed below and record these on the Prescription and Monitoring form.

If you have any concerns regarding the CADD Solis pump, please contact the Palliative Care department via Ext 72551 or via switch after hours and on weekends.

Nursing Observations

For new infusions or medication/dose changes -

- Assess pain, nausea, sedation, subcut site, battery, reservoir volume and bolus dose/attempts, 1 hour after the infusion has been commenced and then 4 hourly observations to be documented on the Prescription and Monitoring form.
- The battery indicator is at the top right-hand side of the display. If the pump shows less than half battery life, please plug into the mains power. Power cord will be supplied.
- The reservoir volume is displayed at the top left-hand side of the screen.

You will need to view “Reports” on the CADD Solis pump to find out the “Bolus Given” and “Bolus Attempts” information. You do not need to stop the pump to view the reports.

1. From the home screen press “Reports”, the button on the left-hand side.
2. The first available report is the “Given and PCA Dose Counters” report. Press ‘select’.
3. PCA Dose given is the figure you record for “Bolus Given”
4. PCA Dose Attempted is the figure you record for “Bolus Attempts”
5. Press ‘Back’ twice to return to the home screen.

Alarms and troubleshooting the CADD Solis Pumps

- **Battery low**, replace battery Alarm Priority Low. Connect the AC adaptor or insert 4 AA batteries soon.
- **Downstream occlusion**. Clear occlusion between pump and patient Alarm Priority High. The pump has detected high pressure, which may be resulting from a downstream blockage, kink in the fluid path, or a closed tubing clamp. Delivery pauses and resumes if the occlusion is removed. Remove the obstruction or stop the pump to silence the alarm for 2 minutes, then remove the obstruction and restart the pump.
- **Reservoir volume low Alarm Priority Medium or Low** (depending on how the alarm is programmed in Admin Settings). Level of fluid in the reservoir is low. Prepare to install a new IV bag/cassette, if appropriate.
- **The screensaver** allows the pump to conserve battery power when not in an edit mode and if no keypad buttons have been pressed for 30 seconds. The pump displays a blank screen. Press any button on the keypad to turn the display on.

For any other alarms or concerns regarding the CADD Solis pump please contact the Palliative Care team on Ext 72551 or via switch after hours and on weekends.

4.0 Subcutaneous Fluid Administration

4.1 Medical Requirements

Subcutaneous infusion and infusion rates are ordered by medical staff and to be charted on the Fluid Therapy and Intravenous Additive Orders form.

4.2 Safety Information

Normal saline 0.9%, glucose 5% or combinations are safe for administration.²

Hypotonic or electrolyte free solutions are not to be used subcutaneously. Most commonly used fluids are normal saline and dextrose-saline.^{2 2}

At SCGOPHCG due to instances of localised oedema contributing to patient discomfort, subcutaneous fluid administration should be by gravity line.

1.5 litres can be delivered at one site, or 3 litres via two separate sites.²

Volumes of greater than 1.5 litres per 24 hours are not recommended.^{2,3}

4.3 Nursing Management

- Assess the condition and integrity of the patient’s subcutaneous tissue prior to therapy.
- Assess for leakage of infusing fluids from subcut butterfly insertion site. If leakage occurs reduce rate of infusion and / or re-site. The subcut butterfly should be checked once per shift and documented in the medical notes.

- National Labelling Recommendations have been developed to assist health care professionals to identify the correct medicine and/or fluid and its correct administration route at all times.
- The **route of administration** must be identified / labelled on **all lines**¹².
- For continuous infusions the giving set and subcut butterfly must be changed every 48 hours.²



4.4 Procedural Information

- Insert subcut butterfly as per Section 1. Insertion of Subcutaneous Catheter.
- Connect prescribed fluids using aseptic technique.
- Document on fluid balance chart

Related Documents

Legislation:

- Health Practitioner Regulation National Law (WA) Act 2010
- Nursing and Midwifery Board of Australia (NMBA) Code of Professional Conduct for Nurses March 2018
- Western Australia: Medicines and Poisons Act 2014, Medicine and Poisons Regulations 2016, Pharmacy Act 2010
- Work Health and Safety Act 2020

Policy:

- Health Mandatory Policy 0131/20 High Risk Medication Policy.
- NMHS Clinical Documentation Policy
- SCGOPHCG Infection Prevention and Control manual -Aseptic Technique Policy.
- SCGH Patient Identification and Procedure Matching Policy

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Authorisation & Version Control

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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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